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Name: سيدہ ابوالعلا محمد ابوالعلا National ID: 27610192101268	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

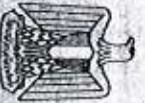
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تعريف الشخصية
 السيد
 ابو الغلا محمد ابو الغلا
 ١٨ اريت رسمي
 شبرا - القاهرة
 ٢٧٦١٠١٩٢١٠١٢٦٨
 ١٩٧٧/١٢
 KQ0211938


 ٢٠٢٤/٠٧ ٢٧٦١٠١٩٢١٠١٢٦٨
 ربة منزل
 ايتش
 مسجلة
 ارملة
 البطاقة سارية حتى ٢٠٣١/٠٧/٠٨



MDT FORM

10/ 9 / 2025 G

Name	سيدة إبراهيم	MRN	2184/2025
Age	48 yrs.	ID	27610192101268
Menopausal status	Menopause.	Phone	01101511013
NRS SCORE		Marital status	Widow.
BMI		Family history	Neg. F.H.
CO morbidity	D.M on med. tu.	REASON OF MDT	New Case.

Clinical Data

BX: (L+) IDC G-II (R) Fibroadenoma

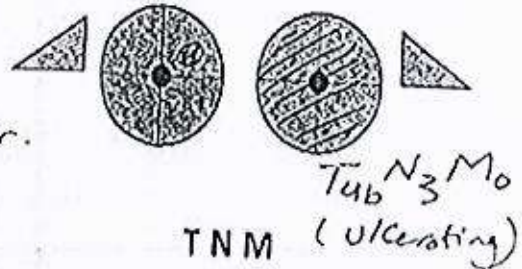
ER: 7/8 PR: 7/8 HER2: 3+ Ki67:

Sonomammography:

→ (L+) Locally advanced, multi centric ipsilat. L.N (L-II) intraclavicular - extracapsular.

Mets Workup:

→ UIQ dense irregular 2.3x1.8x2.3cm.
→ UOQ IMLN 1.1x0.5cm & 0.9x0.5cm
→ Rt lower lung lobe pulm. nodule 0.5cm → (F.U)
→ osteolytic lesions ?? mets.



Recommendations:

Bone scan free. for TV/US → Ca sent on adhxrd cyst 8 weeks
for Neoadjuvant Taxol weekly (12 weeks) + Carboplatin
→ then 9R2H & Rt Fibronelnon
Ex Cesion

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT FORM

10/ 9 / 2025 G

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→ osteolytic lesions ?? mets.					



Tub N3 Mo
TNM (Ulcerating)

Recommendations: for TV/US → sent on adnexal cyst & Uterus
Bonescan → free.
for Neoadjuvant Taxol weekly (12 weeks) + Carboplatin
→ then 9R2H & Rt Fibroneluron
Ex Cession

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name : Sayeda Abul Ela Mohammed Abul Ela

Date : 30-08-2025 12:33

Branch : Dokki

Medical No. : 190030

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 48-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior-whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

PATIENT NAME: SAYYEDAH ABOELELA MOHAMED ABOELELA
 IDER: Female
 E : 19-Jul-2025 11:34 AM

PATIENT ID: 20225021367
 AGE: 48 y
 ORDER: Z221398,Z221399,
 Z221542

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

HNIQUE:

- CC and MLO view.
- Complementary ultrasound

ORT:

- Scattered fibroglandular densities (ACR B).

LEFT BREAST:

- Left breast diffuse edema pattenen in the form of increased skin thickness reaching up to 14mm, coarsened trabeculae & multiple speculated dense masses seen attaching & retracting the nipple & LOQ overlying skin.
- Complementary ultrasound showed multiple speculatyed masses occupying all breast quadrants, the largest seen as follow:
 - Left 12 O'clock para areolar, measuring about 35.3x18.5mm seen attaching & retracting the nipple.
 - Left 1 O'clock zone b, seen measured about 16.7x17.5mm.
 - Left 4-5 O'clock, it measured about 24x18mm, with specules reching & retracting the overlying skin.
 - Left 9-10 O'clock zone a, multiple adjacent lesions, the largest measured about 10x7.5mm.
- Left axillary level (I-II-infraclavicular) pathologically enlarged lymph nodes, some of them at level I showing irregular speculated margins, liukely extracapsular rupture.
- Free supraclavicular region.

RIGHT BREAST:

- No grouped microcalcifications.
- Right UIQ dense irregular mass lesion, it measured about 23x18x23.7mm, it is about 8.8 cm from the nipple, Complementary ultrasound showed right 1 O'clock a microlobulated, hypoechoic solid mass lesion, it measured about 16.6x11mm.
- Right UOQ an IMLN, Complementary ultrasound showed right 9 O'clock zone c & 10 O'clock zone b two parallel, circumscribed, oval shaped, hypoechoic solid lesions with internal echogenicity, likely IMLNs, they measured about 11x5.3mm & 9x5mm.
- No cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.
- Free infra & supraclavicular regions.

IIION:

- Left breast locally advanced, multicentric maignant looking process with ipsilateral

T: SAYYEDAH ABOELELA MOHAMED ABOELELA, 20225021367, Female, 48 y
 version 1, 19-Jul-2025 12:44 PM

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مركز بيهة الهرم
 شارع طه لؤى شارع الهرم بحوار دافى ، العوزة

مستشفى بيهة الشيخ زايد
 شارع الشيخ زايد شارع مكة الهندسة



[DOCTYPE:MAMMOGRAM]

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, and I.V.C.
- Bulky uterus.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Few prominent para-aortic and mesenteric lymph nodes, the largest is seen at the former group measures 24 x 14 mm.
- No abdominal or pelvic masses or collections.
- Divercation of the recti with stretching and thinning out of the linea alba.
- Right adnexal cyst is seen measuring 3.1 x 2.9 cm.
- **The revised bone window revealed:**
 1. Multiple vertebral and bilateral iliac small lytic osseous lesions are seen, the largest is seen at L4 vertebral body measures 5 mm.
 2. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
 3. Bilateral sacroiliitis is noted.

Opinion:

- Left breast neoplastic process with enlarged ipsilateral axillary, internal mammary and supraclavicular lymph nodes.
- Right pulmonary nodule, for follow-up.
- Multiple lytic osseous lesions as described, likely metastatic for correlation with bone scan
- Right adnexal cyst and bulky uterus for ultrasound evaluation.

Reported By,

Dr. Sara Saif, MSc

Dr. Marwa Ahmed, MD

30-Jul-2025 06:46 PM

Patient: SAYYEDAH ABOELELA MOHAMED ABOELELA, 20225021367, Female, 48 y

Report version 2, 30-Jul-2025 06:46 PM

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مركز مدينة النور
4 شارع علوية نول شارع النور بجوار داري . الجيزة

مستشفى مدينة الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة

Ultrasound
[[DOCTYPE:ULTRASOUND]]

PATIENT NAME: SAYYEDAH ABOELELA MOHAMED ABOELELA
GENDER: Female
DATE : 30-Jul-2025 04:20 PM

PATIENT ID: 20225021367
AGE: 48 y
ORDER: Z227571,Z227572

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS WITH TRIPHASIC STUD OF THE LIVER:**

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- **Patient with pathologically proven left breast IDC:
- Diffuse edema and thickening of the left breast showing multiple spiculated mass lesions, the largest of which is seen at the para-areolar region measures about 36 x 22 mm. Diffuse smudging of the left breast fat planes.
- Lobulated right breast UOQ mass lesion measuring about 13 x 14 mm.
- Multiple enlarged left axillary (level I and II) lymph nodes, the largest measures 13.5 x 11 mm.
- Few enlarged left internal mammary lymph nodes, the largest measures 11 x 7 mm.
- Multiple prominent left supraclavicular lymph nodes, the largest measures about 8 mm in diameter.
- Right lower lung lobe posterior segment pulmonary nodule measuring 5 mm.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

PATIENT: SAYYEDAH ABOELELA MOHAMED ABOELELA, 20225021367, Female, 48 y
Report version 2, 30-Jul-2025 06:46 PM

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مركز نهضة اليوم
شارع طلحة أول شارع اليوم بجوار دارى الجيزة

مستشفى بريدة الشيخ زايد
الحي الأول - الشيخ زايد أمام كلية الهندسة



Patient Name: SAYYEDAH ABOELELA MOHAMED A	Gender: Female
MRN: 20225021367	Ordered By: Mariam Ahmed Ibrahim Ahmed
DOB: 19/10/1976 Age: 48 Y 9 M 11 D	Order ID: 221398

2D mammography

Procedure Date: 19/07/2025 11:22:00 AM

port:

port:

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

CHNIQUE:

CC and MLO view.

Complementary ultrasound

PORT:

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LEFT BREAST:

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ormal skin thickness and contour.

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onspecific axillary lymph nodes

AS
SY
/A

Patient Name	الأستاذة / سيدة ابو العلا محمد ابو العلا	Request No.	: 13006398
Patient No.	: 27610192101268	Request Date	: 09/05/2026
Path No.	: 1406/26	Verification Date	: 31/05/2026
Sex	: Female	Financial Category	: نفقة الدولة - جوسفان روسي
Age	: 49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature of Specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 30x18x5 cm, with covering skin ellipse measuring 25x15 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a tumor bed site 7x6x3 cm, located 2 cm away from the deep margin.
- * Axilla: Axillary fat entangling 11 lymph nodes, the largest measured 1x1x0.5 cm.
- 13Blocks:
- 1:7- T.B.S., 8- N&A., 9-Deep margin., 10:13- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive parenchymal tumor.
- Cellularity (0%).
- * Ductal carcinoma in situ: Absent.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive viable metastatic deposits in one out of eleven lymph nodes (1/11) (The metastatic focus measuring 0.2 cm, without extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:

- No residual viable remnants of the previously diagnosed invasive parenchymal tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.

Verified by :

Dr.Duaa Mohamed

Printing Date : 14/06/2026 09:43:54

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt